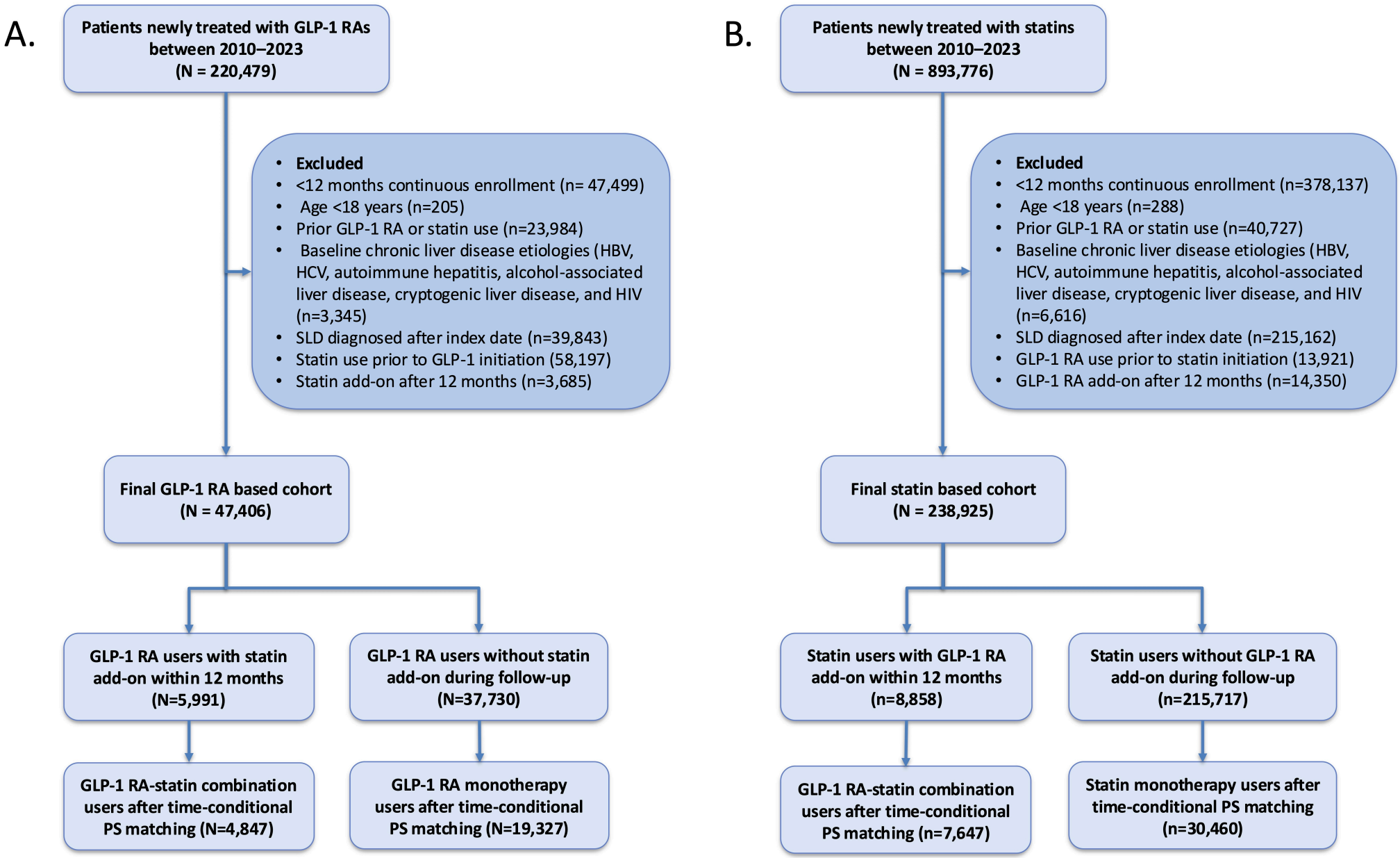

A.
Patients newly treated with GLP-1 RAs between 2010–2023 (N = 220,479)
Excluded
<12 months continuous enrollment (n= 47,499)
Age <18 years (n=205)
Prior GLP-1 RA or statin use (n=23,984)
Baseline chronic liver disease etiologies (HBV, HCV, autoimmune hepatitis, alcohol-associated liver disease, cryptogenic liver disease, and HIV (n=3,345)
SLD diagnosed after index date (n=39,843)
Statin use prior to GLP-1 initiation (58,197)
Statin add-on after 12 months (n=3,685)
Final GLP-1 RA based cohort (N = 47,406)
GLP-1 RA users with statin add-on within 12 months (N=5,991)
GLP-1 RA users without statin add-on during follow-up (N=37,730)
GLP-1 RA-statin combination users after time-conditional PS matching (N=4,847)
GLP-1 RA monotherapy users after time-conditional PS matching (N=19,327)
B.
Patients newly treated with statins between 2010–2023 (N = 893,776)
Excluded
<12 months continuous enrollment (n=378,137)
Age <18 years (n=288)
Prior GLP-1 RA or statin use (n=40,727)
Baseline chronic liver disease etiologies (HBV, HCV, autoimmune hepatitis, alcohol-associated liver disease, cryptogenic liver disease, and HIV (n=6,616)
SLD diagnosed after index date (n=215,162)
GLP-1 RA use prior to statin initiation (13,921)
GLP-1 RA add-on after 12 months (n=14,350)
Final statin based cohort (N = 238,925)
Statin users with GLP-1 RA add-on within 12 months (n=8,858)
Statin users without GLP-1 RA add-on during follow-up (n=215,717)
GLP-1 RA-statin combination users after time-conditional PS matching (n=7,647)
Statin monotherapy users after time-conditional PS matching (n=30,460)